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Adopting a hospitality lens for designing mental healthcare at home

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Abstract: Increasingly, there is a shift toward bringing services that were originally provided in hospitals into the home. Healthcare designers have been supporting this movement by designing medical devices and home care services. However, there is a risk that such shifts simply medicalize the home and erode the valuable informal practices of care that already occur there. Using a research through design approach, we adopted a hospitality lens to understand the rituals of hosting at home and identify potential areas of hostility as mental health consultations enter the home in Norway. This research demonstrates the value of adopting a hospitality lens when designing healthcare at home and how mapping rituals can contribute to a reflexive practice for both healthcare designers and clinicians.

Keywords: mental health; healthcare design; hospitality; home care; reflexivity

1. Healthcare design from the hospital to home

In the last years, a significant effort has been made to move healthcare from hospitals into the home in many Western countries. Designers have played a significant role in realizing this vision by shaping services and products that enable this transition. For example, designers have been involved in designing remote care technologies and medical devices for the household context (Mincolelli et al. 2019; Rinaldi & Tossi, 2017) or service pathways for patients treated at home (Royer, 2019). However, research also highlights that hospitals are constituted by an institutionalized dimension, which includes invisible and unquestioned logics (Delanoë-Voeux, 2022). At the same time, there is recognition that there are many, often conflicting logics of care (Mol, 2008). In bringing the hospital into the domestic context, there is a risk that the design of remote and home care inadvertently contributes to

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eroding the practices and routines of care that are already occurring in home, family, and community life. Despite their good intentions, without critical reflection, designers may be crafting new products and services that end up hospitalizing the home by imposing the logics of institutionalized care onto and into the home and obstructing important rituals of everyday life. Some critiques have already emerged, underlining the potential risks of imposing devices into daily habits. Among others, the work of Superflux studio with the video *Uninvited Guest* shows the constraints generated by such smart devices on an aging man's routines (Jain et al., 2015).

In order to protect the existing routines of care that are happening within home, reflexivity is needed among healthcare designers. Reflexivity involves an in-depth understanding of the context and one's own position within it (Vink & Koskela Huatori, 2022). Recent research highlights the importance of building a nuanced understanding of the social structures within a given context (Vink & Koskela Huatori, 2021), the psychological backgrounds of those involved (Raman & French, 2022), as well as working more mindfully within the multiplicity of logics of care within design research (Nilsson et al., 2022). Furthermore, scholars call for critical approaches to design practice that question possible ways of designing and acknowledge the potential benefits and damages that could occur during the design process (von Busch & Palmås, 2023).

Against this backdrop, we conducted a 9-month design research project investigating potential changes in the hospitality rituals taking place within the home, particularly as psychiatric and psychological consultations shift from hospital to online consultations or home visits in Oslo, Norway. Here we build on work done in ritual design that sees rituals as fixed sets of actions or words performed repeatedly on specific occasions, which are recognized as helpful elements to attend to when designing (Matthews, 2017). To support this inquiry, four background interviews and eight semi-structured interviews were conducted, seven visual maps of hospitality rituals were developed, and a reflective workshop with healthcare practitioners was held.

Through this process, we aim to developed a nuanced understanding of hospitality rituals within the home. We examine how conducting psychiatric consultations online or at home adheres to and shifts regular rituals in the home, with the potential for creating hostility. In doing so, this design research project brings forward a conceptual lens and practical approach that may aid design researchers and designers to critically reflect on how remote and home care products and services can be designed in a way that respects traditional practices and routines of care in households and communities.

2. Adopting a hospitality lens

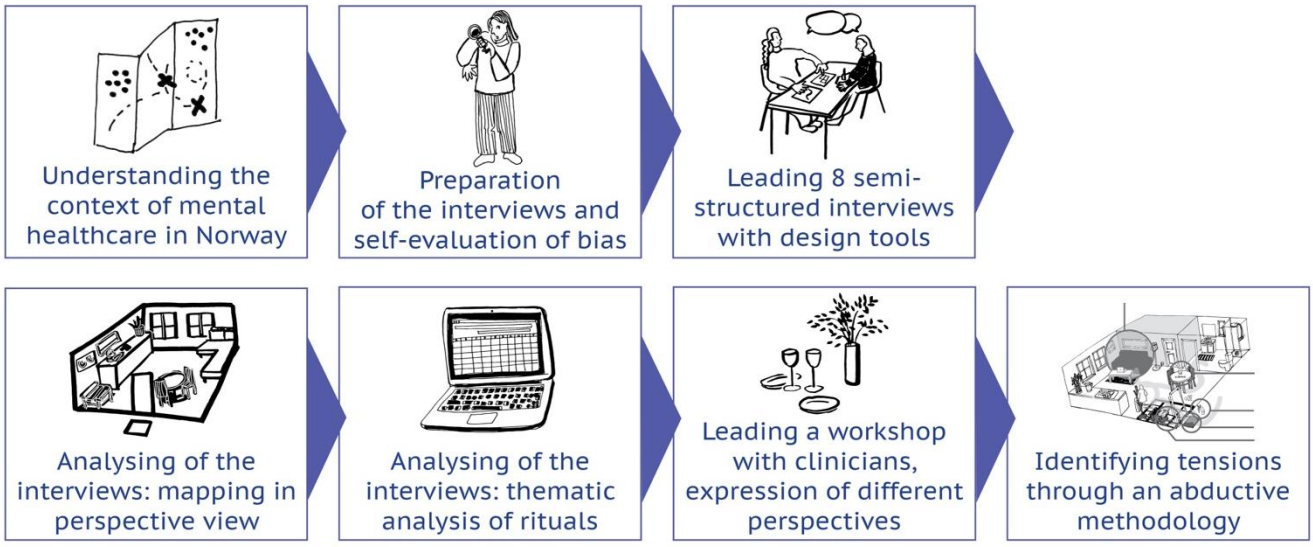


Figure 1 Steps of the project methodology

Through the examination of a particular case focused on bringing psychiatric consultations into the home, this research unpacks the specificity of rituals at home through a hospitality lens. The main steps of the overall project methodology are outlined in Figure 1 and explained in more detail below. Our main goal is to shed light on the importance of understanding rituals and specificities of the household context before adapting care from hospital services or imposing medical devices within the home. Using this theoretical framing of hospitality, we compared three concrete consultation situations: home consultation, remote consultation and consultation at the hospital to a reference situation of hosting a friend at home in Norway. This comparison allows us to identify shifts of hospitality rituals from the home to the hospital and to highlight different dominant logics of care at play according to the consultation location, as shown in Figure 2. Home visits have been recognized as carrying potential to enhance the relationship between clinicians and patients by enabling professionals to construct a more holistic understanding of the client or patient within the context of their living environment or by opening-up for new developments in the therapeutical relationship that are enabled by a more familiar context (WHO, 2022). However, the shift in environment can also challenge these relationships, which demand constant balancing between distance and proximity (Hochwarter, et al. 2023).

The hospitality lens was adopted to help us understand the particularities of the home context in comparison with the hospital. Being hospitable is a basic act of sociability but relations that arise from it, between guests and hosts, are highly complex, made of implicit obligations, coded norms and performed rituals (Sarthou-Lajus, 2008). Hospitality comes from the latin word “hostis” which means at the same time “the host”, “the guest” and “the enemy” ; hostis is also the root of the term “hostility”. Hence, hospitality is fundamentally intertwined with hostility. Derrida’s philosophy makes clear that potential violence is always

residing in acts of hospitality, and, thus, these two concepts must be studied simultaneously (Dufourmantel, 2008; Derrida & Dufourmantel, 1997).

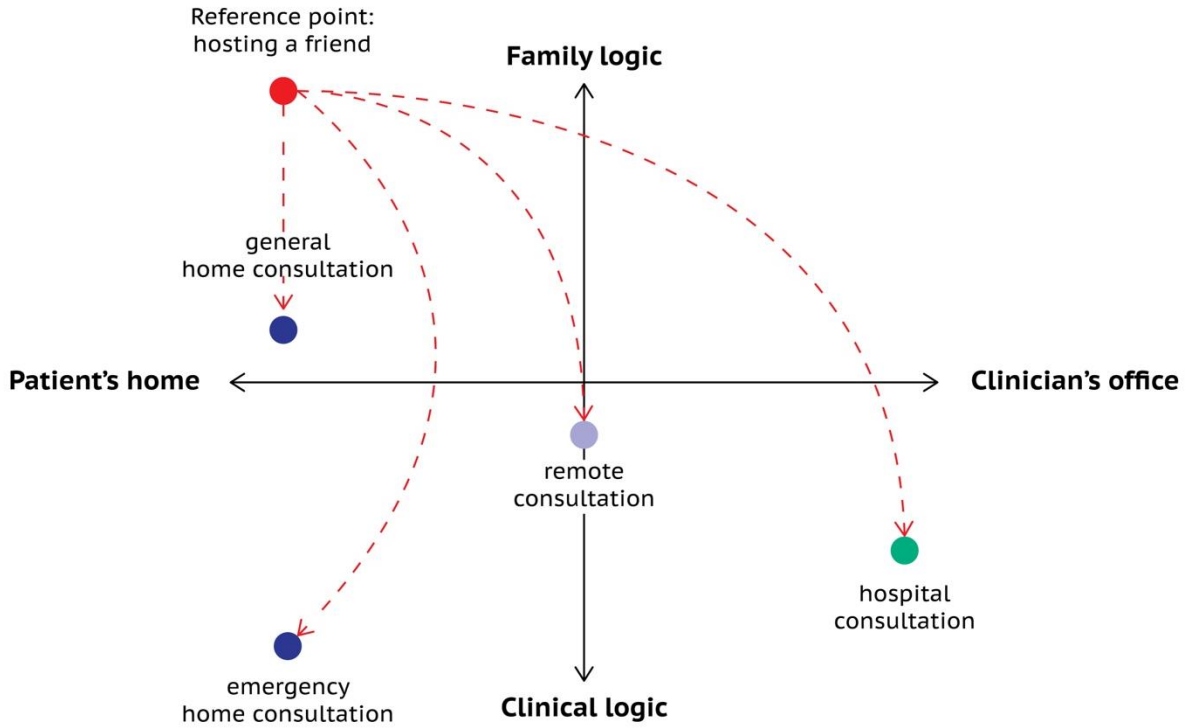


Figure 2 Matrix of the dominant logics of care for different visits based on location

If hospitality refers to the way a person is welcomed into a space, it takes on a specific character in each distinct space. The home and hospital practices enact very different forms of hospitality. Within the hospital, the heritage of hospitality comes from the unconditional welcoming of the hospice (Delanoë-Vieux, 2022). Whereas at home, hospitality is conditional and stands on a pact of trust between hosts and guests (Derrida and Dufourmantelle, 1997). A respectful welcoming into the home by the host, implies a tacit agreement from the guest to not self-impose, follow the house rules, and renounce any pretensions of authority (Montando, 2008). Hospitality at home is guided by an imaginary dimension of meaning and values that are connected with the subjectivity of each inhabitant (Ennuyer, 2017).

2.1 The context of mental healthcare in Norway

This research through design project was conducted in Oslo, Norway, between October 2022 and June 2023. The project was connected to the Centre for Connected Care (C3) research consortium. The research team was constituted by a Norwegian psychiatrist, Norwegian psychologist and two design researchers who are not natives of the country. First, an in-depth understanding of the context of psychiatry in Norway today was sought by the first author who analyzed the history of psychiatry in Norway based on a literature review and museum visits, and synthesized it in a visual chronology. Furthermore, the first author mapped the organization of the healthcare system in Norway, as well as read about Norwegian society and the way Norwegians relate to their homes and close environments. Methodologies of

systemic design were applied to aggregate and make sense of the materials. In particular, GIGA-mapping methods were used to visualize the system from different points of views (Sevaldson, 2022). The annotated map in Figure 3 shows examples of the learnings and questions that emerged through the systemic design work done to inform the contextual understanding of the project.

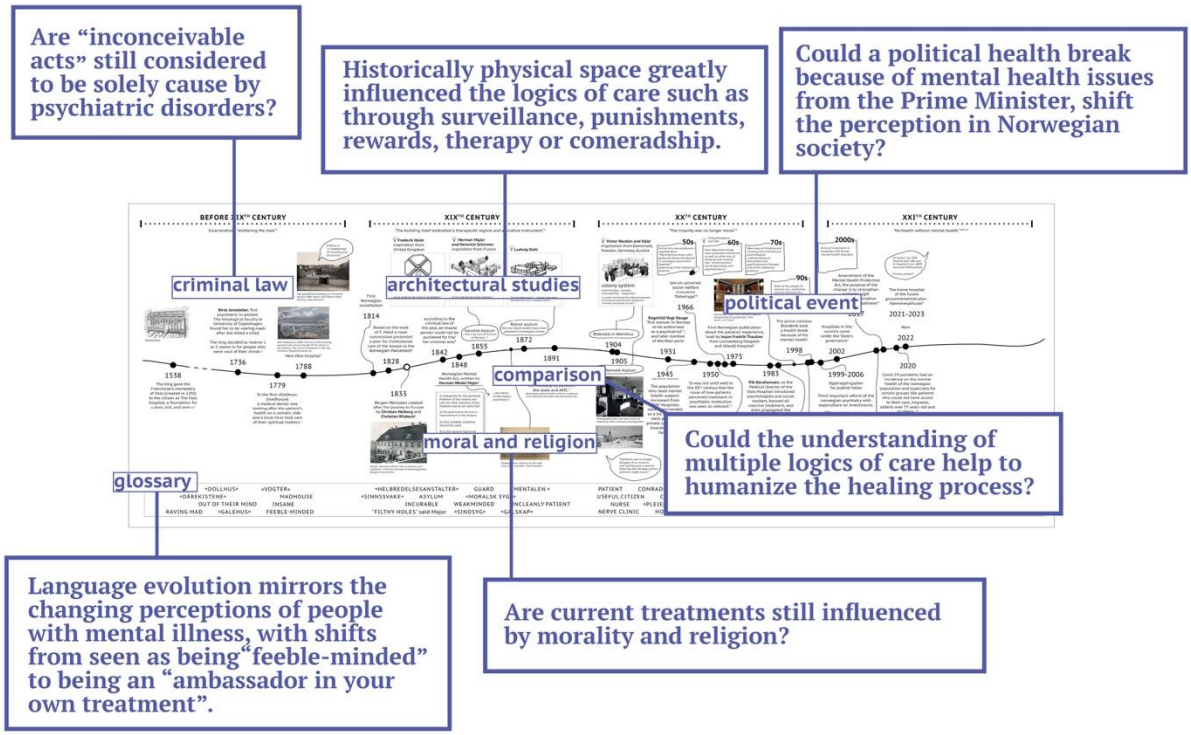


Figure 3 Brief chronological map of Norwegian psychiatry and examples of emerging learning and questions

2.2 Background interviews about remote consultations

The clinical team, consisting of a psychiatrist and a psychologist, conducted and translated from Norwegian to English four interviews with three clinicians and one person with lived experience of receiving specialized mental healthcare (shown in Table 1). This was part of an exploration to understand expectations regarding the set-up of remote consultations in mental health care. The discussions were analyzed and we extracted some initial concerns and questions to elaborate on an interview guide for the next phase.

2.3 Semi-structured interviews about home rituals

Table 1 Details of participants in the research

Background interviews	Semi directive interviews	Workshop

2 clinicians	4 Norwegian hosts	2 clinicians
1 person with lived experience	2 psychologists	1 psychologist
1 unit leader	1 psychiatrist	1 psychiatrist
	1 person with lived experience	3 designers

This second phase of interviews was dedicated to understanding the pathway of hospitality in Norwegian homes and to comparing these with habits of remote, hospital or home consultations led by clinicians, in order to inform specificities of consultations in a home context. The initial part of the interview process was an auto-analysis of the interviewer to open-up the bias, beliefs and norms that one may convey, as theorized in sociological publications (Beaud, 1996; Scarfò Ghellab, 2015). Over a 6-week period, we conducted semi-structured interviews with 1 psychiatrist, 2 psychologists, 1 person with lived experience as a service user of mental healthcare, and 4 Norwegian residents (shown in Table 1). Each interview was conducted in English and lasted approximately one hour.

During discussions, interviewees were asked to explain how they welcomed or were being welcomed in the home both spatially and chronologically. The questions were based on prepared elements to help participants express themselves through drawing, sequence and qualification of the hospitality moments. The supporting tools produced and used were an A3 timeline with thematic cards to glue on it, such as: greeting, farewell, polite manners, and an A3 white paper with pencils for the interviewee to map their home while explaining their own way of hosting. A few carbon papers were also available in order to draw other distinct pathways, such as a nurse's or a doctor's visit. Thus, the hosting moments were looked at through the rituals that they were composed of from a narrative, spatial and chronological perspective.

2.4 Analysis of the interview: Mapping

During the project, we constantly materialized the various situations shared or experienced so it became a support for discussion. Several tools were used like frameworks, data visualizations, and maps. To be able to share insight from each interview, the first author designed a map of the hosting moments described by each interviewee. An example is shown in Figure 4. The "perspective drawings" were preferred to a "plan drawing", in order to make it easier to read. Each map was checked afterwards by the interviewees to confirm the interpretation of the plan they drew during the interview. By drawing the event of hosting, it gives importance to small details that may be valuable, but often ignored. We worked to make visible all of the sequences of ritualized moments in the consultation process studied (Aguirre et al., 2017). The aim is to shed light on "what goes without saying", raising awareness of the purpose of activities of hosting in another context. The maps worked as tools to support the unpacking of habits that were to be analyzed afterwards. Only seven of eight interviews were mapped like this, as one participant was not able to contribute in this way. All

interview recordings were transcribed through a digital transcription tool, re-read and adjusted manually thereafter.

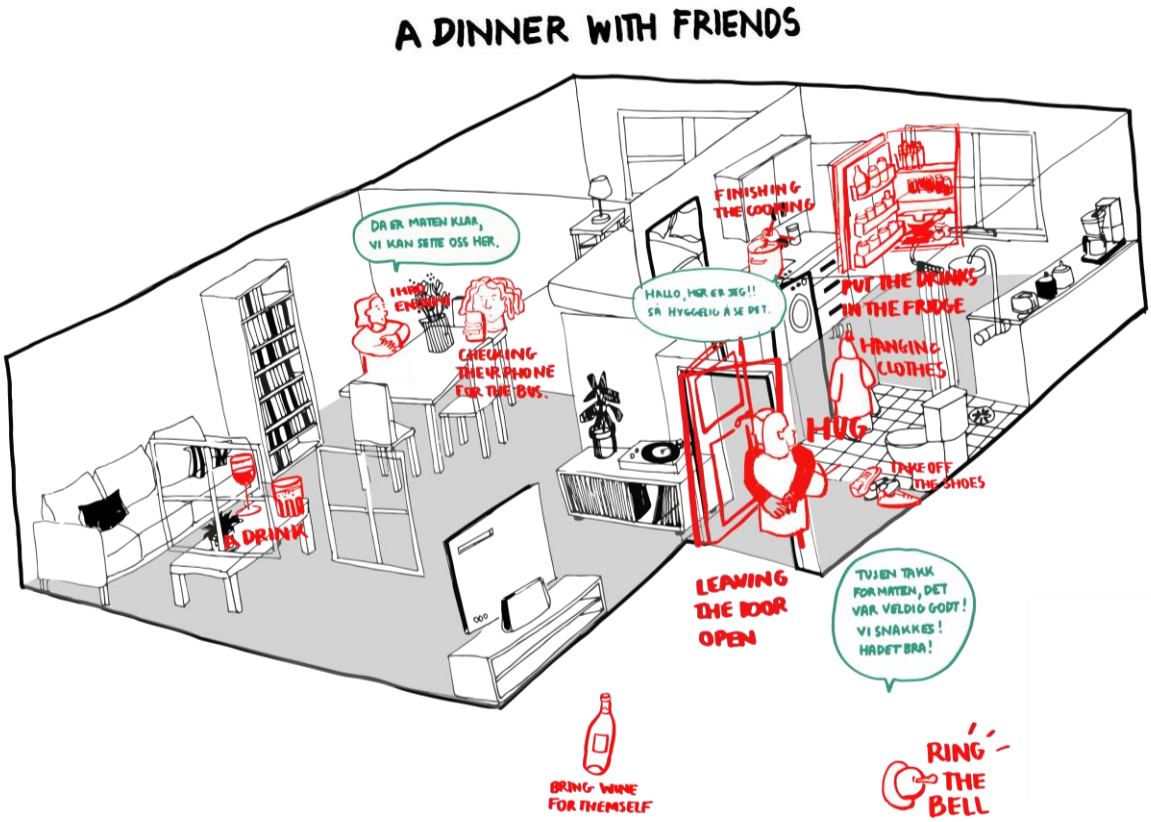


Figure 4 Perspective view of the interview summary of one host

2.5 Thematic analysis of rituals

After the analytical map, we focused on some transversal aspects like objects triggering hospitality, commonly used words when people meet, how guests move through a home or the quality of different spaces in the home. We used specific maps to compare different practices at play through the four contexts. A lot of rituals in entering a patient's home or entering the office of a clinician were based on the space and the objects there. These objects played a key role in supporting hospitality or even triggered it. In parallel, we used an excel sheet to do a first coding of the data which helped us to notice some first emerging themes.

2.6 Workshop with clinicians

The analysis of background interviews of the clinicians and the 8 interviews about rituals in the home informed a workshop at the healthcare innovation conference, which focused on hospitality and hostility during remote and at home consultations. During the workshop, we worked on unpacking hospitality and hostility moments through the narration of a story. The story was made from the synthesis of the interview material and was accompanied by a map to help participants to examine each moment in context. We proposed to role-play some

key situations enacted by participants in front of the rest of the group. These activities were discussion starters between the clinicians and designers, helping us to understand the different practices in homes and offices related to the service as well as differences between service users. The discussions allowed us to nuance our comparative understanding between friendly visits, consultations, and other mental health services at home.

2.7 Abductive approach to identify tensions

After the workshop, we continued to analyze thematically by crossing the workshop material with the material from the eight semi-structured interviews, and the first four background interviews (Thomas, 2006; Lahire, 1996). Through a qualitative and thematic analysis, we continued to refine our understanding of common rituals by clustering them by theme and organizing patterns of ritual flow (Brooks et al 2015). By comparing pathways of hospitality at the hospital and at home, we noticed some overlapping moments that had similar rituals but with different intentions. We then identified tensions, which are areas of existing frictions or possibilities for hostility within these rituals. In parallel, we used philosophical concepts to shed light on and enhance our findings, using an abductive approach (Redström, 2017). Based on this analytical process, there are two main outputs from this work: first, we constructed a nuanced understanding of hospitality rituals in the Norwegian home; and second, we identified several tensions and selected three representative ones from the comparative analysis to unpack the potential for hostility in hospitality rituals when shifting from the hospital to the home.

3. Patterns of hospitality at home in Norway

3.1 A shared pathway for hosting

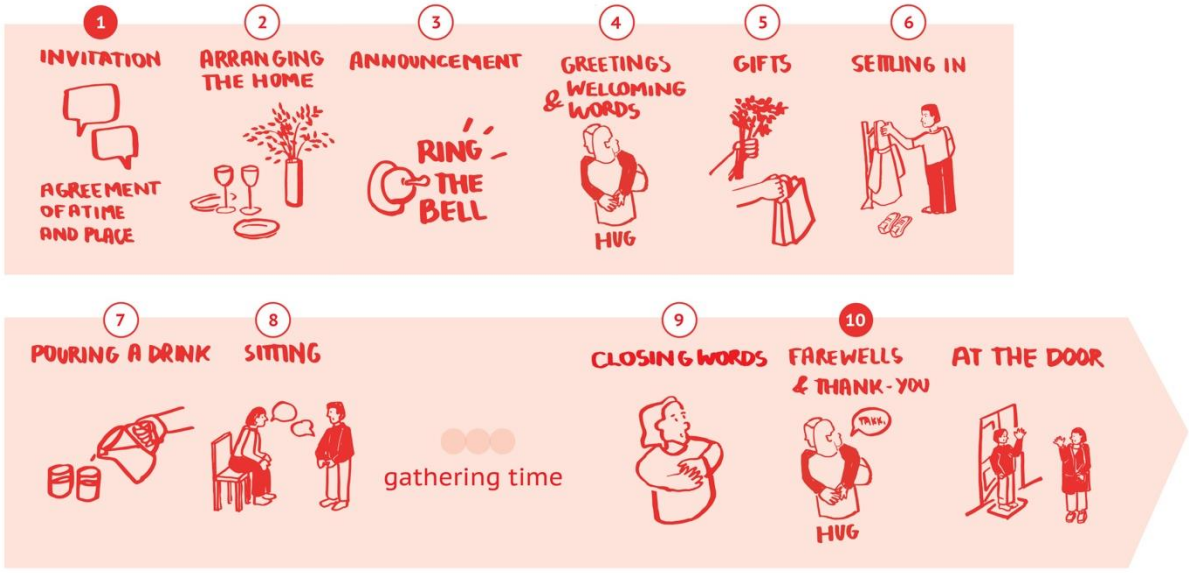


Figure 5 Ten main situations of hospitality ritual flow when hosting a friend were identified as repetitive patterns

Ten main situations were identified within an overall flow of typical hospitality rituals performed when hosting a friend. These situations generally occurred around a central dinner and spanned from before the guests' arrival to the guests' departure. The situations are as follows: (1) the invitation—confirming the place and date in advance, (2) preparation—arranging the home for guests' visit, (3) announcement—ringing the bell, (4) greetings and welcoming words, (5) gifts—giving or bringing something,(6) entering the place, hanging clothes and taking off shoes, (7) pouring a drink and snacking on starters, (8) colloquial sitting before dinner, and at the end of the evening, (9) closing words, and finally (10) farewells and thanks. Each of these situations are described in more detail in Table 2 below, supported by quotes.

Table 2 Situations within the shared pathway of hosting in Norwegian homes

Situation	Quote from the interviews
1. Invitation Most frequently, the hosting moment was planned in advance. The guests received an invitation a few days before the event. One host highlighted the invitation as an integral part of the host's role.	Host: "But I think it's something you have to practice to be a host. (…) So the role is to invite people over a few days ahead, so they can know that they are expected."
2. Arranging the Home for a Visit At the agreed time, the host is waiting for the guests to arrive and has prepared for their arrival in advance. The arrival time for dinner is around 18h or 19h and occurs more often during weekends. The level of preparation is different according to the hosts personality.	Host: "I try to prepare everything in advance so that I can sit and enjoy as much as the guests. And I always prefer to not do too much cleaning or arranging during their stay."
3. Announcement of Arrival At the time of their arrival, guests made themselves known by knocking or ringing the doorbell.	Host: "'*Koselig*': 'It's nice to see you'. It's a Norwegian sentence that is a way of greeting. '*Kos*' is a very Norwegian expression and with the cuddly kind of feeling."
4. Greetings and Welcoming Words After hugging, hosts and guests exchange welcoming and greeting words such as: "So nice to see you".	Host: "'*Koselig*': 'It's nice to see you'. It's a Norwegian sentence that is a way of greeting. '*Kos*' is a very Norwegian expression and with the cuddly kind of feeling."
5. Gifts, Giving or Bringing Something Depending on the age of hosts and guests, they will bring different items to the gathering. Young people will often bring their own alcohol so the host does not have to provide for it. Adults with more stable incomes will often arrive with gifts for the host.	Host: "They bring wine, and sometimes they may bring flowers. And then you say it is a gift to the hostess: '*vertinnegave'*."
6. Entering the Place Guests take off their jackets and shoes as a way to materialize their entrance in the host home. By this action they install themselves physically by getting comfortable. Different practices are specified by hosts, some guests bring clean shoes for the inside, some stay just in socks and others accept slippers offered by the host.	Host: "Some wear their socks, but some bring shoes to wear inside. (…)Proper shoes that you can wear inside, that's not the dirty boots that you have outside."

Situation	**Quote from the interviews**
7. Pouring a Drink and Having Starters Once they are feeling at ease to stay a while inside, guests are welcomed with a drink and some snacks before being seated at the dinner table. This informal moment allows the host to finish cooking and preparation while guests are having a drink. They may also help with the last preparations, such as cooking or lighting the candles.	Host: "And usually I serve some snacks or something to drink around this (…) standing table. (...) And we're standing there quite for a long time. Because you serve and drink and you talk and you catch up."
8. Colloquial Sitting Before the Dinner Often this relaxed talking moment before the served dinner takes place in another area of the home, the couch area or around a standing table next to the kitchen. So when the dinner is ready the host will announce it and ask guests to move from this place to the dinner table.	Host: "It's not that fancy often. So like, for starters, maybe just a drink (...). So it's just like maybe the sofa area is maybe more a place to wait before the dinner is ready".
9. Closing Words After the dinner, when they want to leave, guests would announce it verbally or have a look on their phone to check the bus timetable. During the interviews only one person explained how to allow oneself to leave before their guests because it was a very long friendship and their partner would stay. Otherwise, hosts explained that they wait for their guests to suggest when to leave.	Host: "You know, it's always those who want to leave who say: 'We have to get home'. I never say: 'You have to leave'."
10. Farewells and Thanks The final moments involve gently moving from the dinner table to the entrance room to put on shoes and coats. Guests thank the host for their hospitality, complementing the time together or the food before leaving.	Host: "They say (...) sometimes: 'Takk for meg', 'Thank you for me or us' . (…) And sometimes they say: 'This has been so 'koselig', again, or: 'The food was wonderful.' Or 'This was so nice. We have to meet again soon'. (…) And usually they don't all leave at the same time."

3.2 Observing possibilities of hostility

From the interviews, we understood that Norwegians most often host dinner with really close friends. Dinner with acquaintances was not mentioned. On the contrary, the interviewees explained the habits of having a group of friends with whom they invited each other over, each taking turns hosting. In that sense, awkward moments were very few in the host stories, and often justified by the quality of the relationship between them and guests, but

some occurred nevertheless. For instance, during a dinner one host explained that the quantity of the main dish was too low for the number of guests and that guests ate even less:

> "It's actually the opposite here, if you take twice, you can come off as a bit that you take more than you need.(...) I made this soup for my friends a few weeks ago and it was obviously not enough because I didn't manage to calculate. And then I noticed that they were really eating not much at all. There was even soup left in the casserole when they left. And I think, that's kind of a way to show politeness (…) a politeness from the guest to the host to say: 'You've done enough'."

Another situation that could be uncomfortable for the host is when guests enter intimate spaces without asking for permission, such as the sleeping room or open some closets. One host expressed what would be their reaction:

> "Yeah, that could be uncomfortable if they were spending a lot of time in my room without asking. (…) I can understand if they would go there(…), for example, you get a phone call, there's like the only place in my apartment where you can actually close the door (…). So if it's just to go there and close the door and he or she hasn't told me anything and just staying inside, I think I would wonder."

Other actions like refusing to take off the shoes, not saying thank you after the evening or not bringing anything with you after a formal invitation, could sometimes be considered as rude and create discomfort and an uneasy situation for the host and guests.

4. Potential of hostility in care at home rituals

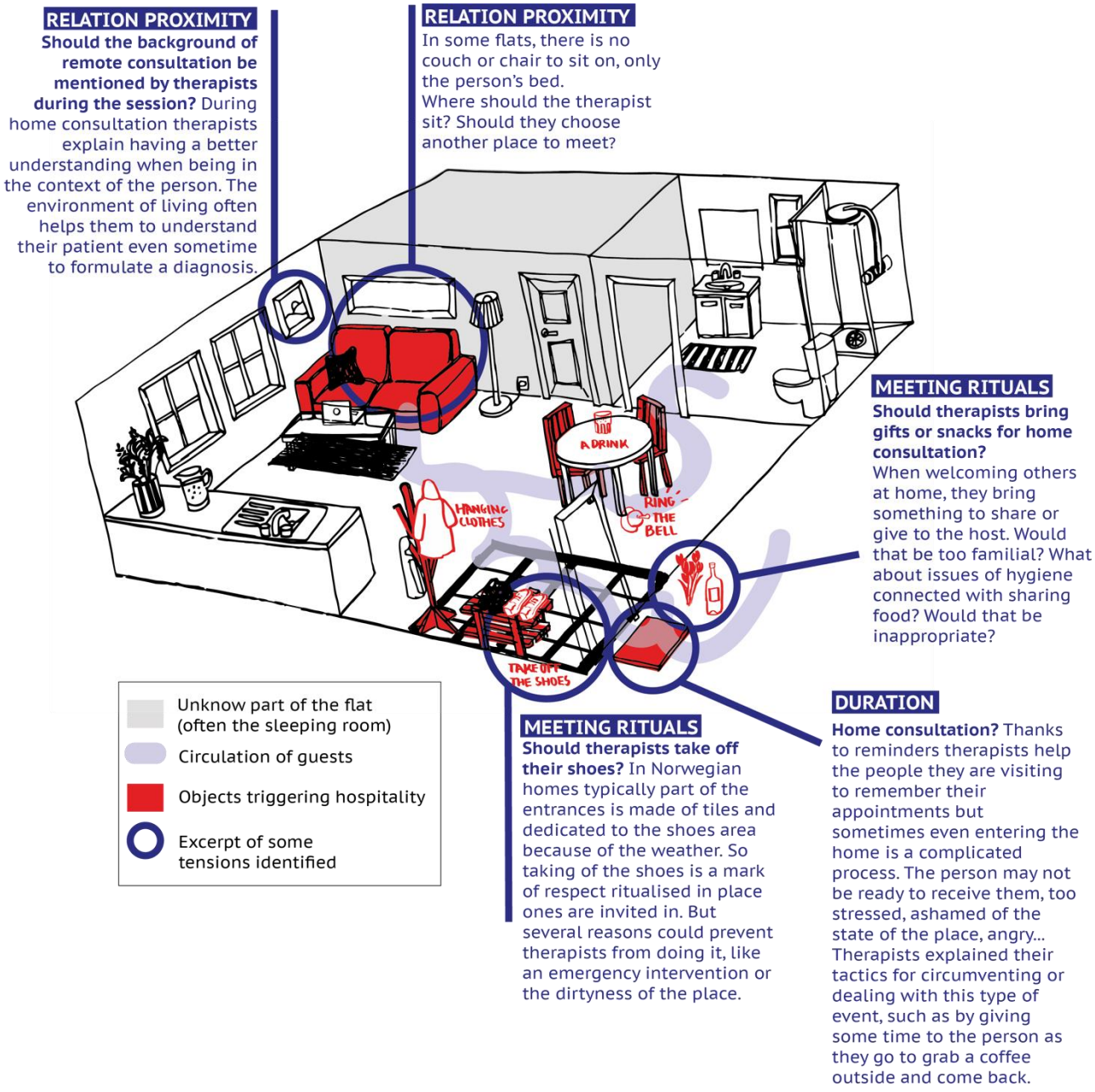


Figure 6 Spatialization of tensions identified visualized on a generic apartment layout

Hospitality is a concept explored notably by the French philosopher Jacques Derrida. He describes hospitality as an act in which violence is intrinsically intertwined. Rober and De Haene (2017) explain the term of violence as used by Derrida as: "(…) subtle and unintended, and is an inevitable aspect of the encounter between myself and the other.". According to Derrida's view on hospitality, there is always a residual part of violence in a meeting between individuals.

We noticed that rituals of welcoming a friend at home and welcoming a patient into the clinician's office have some shared proceedings. However, these overlapping rituals do not share the same intentions or characteristics due to the kind of hospitality they relate to. Even in the home, we identified repetitive ritual patterns that could appear similar in the way they are performed, but yet are based on different intentions according to the situation at play. This is especially true when these rituals occur in a familiar, everyday context versus a therapeutical one, such as the discrepancies shown in the example in Figure 7. This can create friction in places where such rituals collide, for example, in care at home. Indeed, there is not a complete shift from one paradigm of hospitality to another but rather a negotiated coexistence between both of them. Several tensions arose within context of remote or care at home, illuminating potential areas of hostility. The rituals that were adapted to a particular situation in hosting a friend at home, or in a consultation at the hospital, in another context could create a hostile situation. From our observations, rituals of hospitality are determined by a lot of specific factors in which they occur. Interfering with this ritual process can create tensions in the realization of the ritual, corrupting its meaning, legitimacy or even authenticity.

Based on the analysis, we will present three main tensions, as potential areas of hostility, highlighted in Figure 6. These are chosen because they are adaptable to other medical contexts and could exemplify diverging rituals between hospitals and homes relevant for designers. The first is related to the duration and the mastering of time. The second is linked to relational proximity and how this collides with professional distance. The last is regarding the balance between everyday life and the sacredness of a therapeutic consultation.

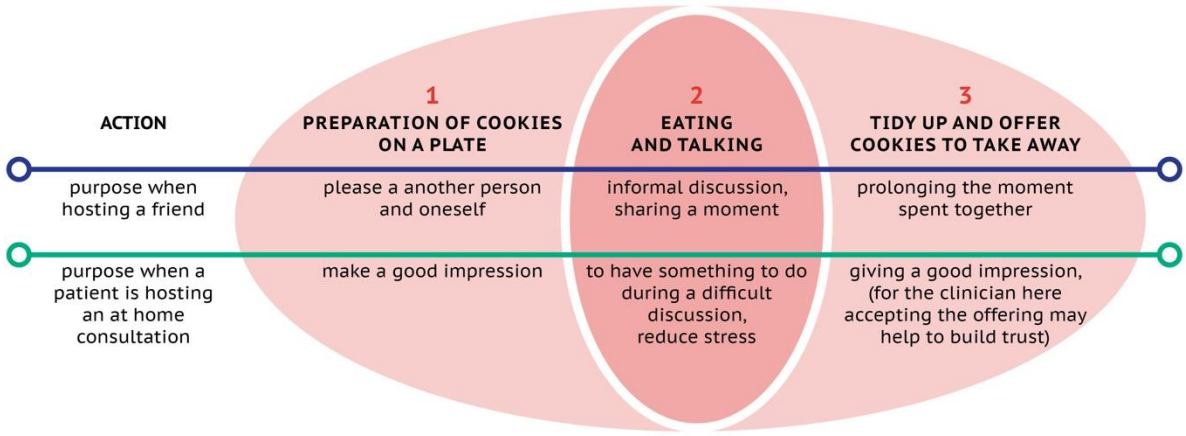


Figure 7 Example of diverging purposes while performing the same rituals in two different settings (diagram adapted from Matthews, 2021)

4.1 Duration: Intuitive or standard

Hospitality rituals around the mastering of the time are quite implicit. Often interviewees explained waiting for the guest to express their will to leave. Indeed, the relation to time when hosting relatives tends to be very intuitive. There is a presumption that guests will leave (Sarthou-Lajus, 2008). However, the right time to go is rarely written in advance; it should

not be too soon, so as not to vex the host and not too late, so as to respect them and not abuse their hospitality. This intuitive way to relate to time collides with the regulated time of clinicians who should anticipate the next person that they must visit or their transportation and journey time.

> Clinician: "It crushes [me] when he stops it at 45 minutes. (…) You need to leave them some time, if they are angry, you go out for five minutes and then you come back. So they could feel that they are met."
>
> Former patient: "(...) [If I were a clinician, I would never] tell a patient: 'Now you need to stop talking.'. Which leaves you kind of brutally hanging in the air."

These two quotes show that, while at the office, it is well accepted to conform to the regulatory 45 minutes (Oslo Universitetssykehus, 2016). However, it could create hostility and violence when imposing these same norms in the home, where procedures around time are often more intuitive and free-flowing. Each quote expresses the brutality of not feeling met at your own place and of stopping abruptly the flow of a consultation. Moreover, the home is a place to live, but also a place of being (Heidegger, 2001). The patient's home is, therefore, the place that responds to their own rhythm, tastes and way of life. The clinician intervenes there as a visitor and even as a guest. The expectations of the person being visited are different from their expectations in the office and any frustrations could have a bigger impact on the person in their own home (Ennuyer, 2017). By leaving a person at home with the feeling of not being met, or being left without a satisfying ending of the conversation, the clinician may create tension in a space that is meant for relaxation, feeling safe or retreating. Thus, imposing a time constraint in this setting can be experienced as an intrusion, or as a way of imposing oneself on the patient.

4.2 Relation proximity: Intimacy or professional distance

Norwegians' homes are typically "closed" as mentioned by one person during the background interviews: "We invite only family or really close friends. If you're invited to a Norwegian home that means you are like family." Furthermore, interviewees often justified not having experienced uncomfortable situations because the people invited were really dear friends. In that sense, entering the home is a privilege of only a few and implies an expectation from visitors at their home to be of the closest connection, a level of intimacy well beyond that which is typical in therapeutic settings.

Patient explaining that remote consultation will only be successful if clinicians engage themselves intimately in the relationship: "This engagement will be the prize, showing personal interest, not just interest in 'healing'."

However, this expectation of proximity in the relationship has an impact on clinician's involvement and emotional labour as shown in the following quote.

> Clinician: "So, for example, I can leave there [the patient's home] and ruminate a little bit. (…) There is a lot of potential and richness in observing or visiting people where they live. The flip side of that, when you work with people that have huge struggles, is that it takes a toll on you as a professional."

This situation creates tensions in the practice and raises questions regarding what is a balanced or appropriate way of acting? The hospitality moments studied in relation to hosting friends were based on sharing an intimate moment around food. In that sense, guests were bringing gifts to show their thankfulness to the host, who in exchange welcomed them around a shared meal, prepared in advance only for them. Some clinicians explained that they were questioning themselves about bringing something to eat to the patient's place. Some mentioned bringing coffee as a gift for the host and also as a nice avoidance of the patient's own offering. "How to refuse an offering?" was a question debated during the workshop. Reflections arose around the balance between caring about professional distance and showing respect to the patient who is hosting the clinician.

4.3 Rituals of the meeting: Consultation or discussion

The home is the place of memory, recollections, and souvenirs (Bachelard, 2012). To enter someone's home is to gain access to a large part of their life via objects, atmosphere, and living space. In addition to being welcomed into the space, guests often help during the hosting moment by participating in some tasks suggested by the host. Some interviewees mentioned even preparing some tasks intentionally left to the guests before a dinner begins. Participation is, in that sense, a part of the welcoming process.

> Host: "So I have lit all the candles in the house, but not the main table. (...) If I have familiar guests and they ask me if they can help. I say: 'Yes, great to light the candles.' So often some of the guests light the candles and help me to pour the wine."

On the contrary, in-office consultation time is completely devoted to therapy. Since it is the clinician's working time, a consultation is generally billed, which implies that the interview should be efficient. The whole space is dedicated to the smooth running of the consultation to ensure maximum concentration: the placement of the seats in the space, not completely facing each other, the clock, and the pack of tissues (Levasseur, 2020). When discussing at home or during remote consultations, interviewees spoke about the almost sacredness of these moments.

> Clinician: "(...) When you're in a psychologist's office, I think your mind is kind of it's set to something(...), something like we're taking it seriously. (...) This time is for you. (...) I've talked to some young people. I think they are super adjusted to video consultations or video calls. So they would be like just putting it [the phone or computer] here and they're like doing stuff even. So they're like not so connected. Of course, those conversations are not therapy sessions. They will be more like advice."

The contradictions within these two quotes highlights tensions between the everyday life of the home and the sacred time of the therapeutic consultation. A home consultation often forces the clinician and patient to navigate between these two polarities. During the workshop, some clinicians shared that they sometimes go to the grocery shop with their clients, because they notice their nervousness before hosting their family, for example. They considered this action as a part of treatment, but this is a significant shift in a theoretical paradigm of what a consultation should be.

Changing the space challenges the fluidity of the therapeutic relationship. This relationship needs to be a constant negotiation based on awareness of hazards, uncertainties, and personalities in order to avoid whenever possible, the emergence of hostility in such an intimate and safe place as the home.

5. Advancing a hospitality lens for healthcare design

Through this research, we demonstrate the value of using a hospitality lens within design practice in healthcare. We further show how ritual mapping can be used as a reflexive practice between patients, clinicians and designers to get a more nuanced understanding of the home context. Below we elaborate on the contributions and implications of this research and note some limitations of this work, which open up for future research opportunities.

5.1 Demonstrating the value of adopting a lens of hospitality

Hospitality is a concept often taken up by philosophers, exploring the hostile duality intrinsic within it (Dufourmantel & Derrida, 1997). Hospitality has also been explored by social sciences studying how its practice influences relationships between care practitioners and people visited at home (Ennuyer, 2017) as well as in care settings of psychological therapy (Rober & De Haene, 2017). We extend this line of inquiry by demonstrating the value of the hospitality lens in design practice that works intentionally with the context of the home.

As highlighted in Section 4, research that adopts a hospitality perspective can aid us in recognizing the many possibilities of hostile situations in healthcare design at home. The entanglement of hospitality and hostility allows us to recognize potential violence in design practice, acknowledging the possibility of hostile situations at every stage of a consultation process at home. This lens can support a more reflexive approach to violence in healthcare design, particularly violence which is not only explicitly brutal but also subtle, unintended and inevitable (Rober and De Haene, 2017).

Furthermore, in this research, we address the ongoing risk of hospitalizing the home by reimposing medicalized ways of caring within the household context. We do so by starting from a home perspective to shift power dynamics within and through design practice. Indeed, healthcare design projects generally take place within hospitals (Huyhn-Dagher, 2022). By working to understand typical rituals within Norwegian households, we get access to highly contextual knowledge of the cultural environment of the home. We were able to map informal practices of care, and non-medicalised ways of caring, so as not to stifle them with already established medical care practices. The lens of hospitality itself, guided us to acknowledge the multiplicity of informal and organic forms of caring and better consider how to support them through clinical care practices (Fisher & Tronto 1990; Royer & Pellerin 2023).

5.2 Moving toward reflexive methodologies

We extend the existing research on rituals in design (Matthews, 2021) by proposing an approach to understand rituals related to hospitality through mapping. The mapping methodology we developed and employed can inform other designers interested in exploring hospitality and hostility moments. In our research, mapping was used for two different purposes. Firstly, during interviews, drawing the plan of the home of the interviewee was a way to allow for mutual comprehension and for the interviewer to grasp the spatial characteristics of the hosting activities. Maps developed in the second phase, supported a translation of the circumstantial understanding related to the interviewee's oral explanations into a shared tool materialized by a “perspective drawing” and the overlayed drawings of the rituals explained.

These maps helped all team members to better access to the interview material and enabled common reflections on related care practices. This mapping approach can also help to open-up and support discussions on care practices between inhabitants and healthcare practitioners by embracing a broader understanding of care than a narrow focus on medical practice. Furthermore, this methodology helped to bring forward the marginalized and invisible forms of care that were otherwise neglected in existing discussions about remote consultations and home care.

In our research, we decided to look for moments of both hospitality and hostility, to be able to show their intertwining or juxtaposition in the maps. This commitment has aimed to raise designers' awareness about their responsibility regarding the implications of the products and services they shape, especially in vulnerable contexts like the home. Through the lens of hospitality and developing a novel approach for mapping hospitality rituals, this research supports the much-needed development of more reflexive approaches to healthcare design (von Busch & Palmås, 2023; Vink & Koskela-Huotari, 2022).

5.3 Future research on the hospitality of healthcare at home

This research was focused on building a nuanced understanding of rituals of hospitality at home. Further research could explore the applicability of this work in other specific care contexts. These findings could also inform the development of tools and apps for remote care consultations, as well as home consultations. Building on these insights, a future project could also explore the best way to establish recommendations or guidelines for healthcare providers when entering people’s homes using a hospitality lens.

We see potential for these mapping methods to be extended in order to develop feedback loops between patients, clinicians and designers to enable a continuous adaptation of healthcare products and services in the home, while being able to discuss together about the rituals of the consultations. Would it be possible to construct or deconstruct the rituals after a therapeutic follow-up with the patient and clinician? Future research could also analyze the effect of the implementation of specific remote care technologies and home care services using a hospitality lens to understand unintended consequences as well as support thoughtful adaptations and interactions in the design.

Finally, our examination of how understanding hospitality and hostility rituals could occur in remote or home care services reflects highly-situated fieldwork, representative only of habits by a select number of Norwegians in Oslo, Norway. Even within such care practices in Norway there is significant cultural diversity and a broad range of hospitality practices. Thus, more research is needed to better understand hospitality rituals at home from a cross-cultural perspective. Future research is needed to develop adaptive tools to enable a cultural-specific understanding of hospitality rituals in order to ensure inclusive and equitable health services across cultural backgrounds.

Our research examined how conducting mental health consultations online or at home adheres to and shifts regular hospitality rituals within the home, with the potential for creating hostility. We offer a first step for continued research into exploring how design approaches may employ a hospitality lens to protect and nurture traditional and informal practices of care during remote or at home consultations.

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